Royal Devon University Healthcare NHS Foundation Trust

Standard Operating Procedure (SOP) for Intensive Care Unit (ICU) Self Sufficient Staffing Model and Redeployment of Intensive Care Staff

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Note: This document has been assessed for any equality, diversity or human rights implications			

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- **1.0 - Flow chart for use at times of low occupancy / acuity**

- Acuity of patients reviewed in Intensive Care. All staff allocated appropriately.

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- All bed spaces ready for level 3 admission.

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- Identification of additional staff to current requirements.

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- Clinical Matron or senior nurse on shift to contact *eligible staff offering flex off hours for the following shift.
- There must always be an immediately staffed level 3 bed.

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- Assess when each nurse last offered flexing off hours to ensure fair opportunity.
- Ensure net hours do not exceed 6 hours before offering flexing off hours.
- Consideration needs to be given to skill mix and acuity of patients (Appendix 2)

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- Contact each eligible member of staff to offer them flexed off hours.
- Staff offered flexing off hours the following day, are required to phone the unit by 0600, speaking to the nurse in charge to confirm it remains an option.
- Where possible the staff member should flex onto a shift within the same roster period
- or they give their availability for paying back
- the hours back within the 6 weeks.
- Document on the self-sufficient staffing model record sheet (Appendix 1) including unit dependency and comments regarding decisions.

- *Staff on fixed Rota's are not eligible to flex off hours due to the inability to repay them.

2. INTRODUCTION

An Intensive care u nit and activity is an unpredictable and dynamic environment, which can change significantly during any shift. This requires adaptable and fluid staffing models to ensure it meets service provision and ensures patient safety.

A flexible working scheme which enables eligible staff to bank hours at times of low occupancy and or acuity, supports this structure. Staff who have banked hours can repay these at peak times, providing a responsive, dynamic workforce, potentially reducing specialist and expensive agency spend during increased occupancy or due to short term sickness.

Redeployment of staff is a last resort and best practice principles should be used to ensure patient safety. Many Intensive Care staff do not have relevant ward experience or the appropriate skills and therefore can only be considered to work in specific areas.

- The provision of the intensive care service has priority over individual needs.
- The final decision lies with the Nurse in charge of ICU
- Staff who bank hours should be up to date with all mandatory training.
- Staff may bank hours at times of low demand and repay those hours at times of high demand or within 6 weeks.
- Using permanent staff to cover peaks will improve safety, potentially reduce agency spend and help to ensure an adequate skill mix.
- The maximum number of hours that can be kept in debit is 11.5 hours not exceeding 6 weeks.
- Intensive Care staff will still work their quota of nights and day shifts to maintain a fair shift pattern for all staff on the unit.
- Staff cannot work bank, overtime, or extra hours if they have outstanding hours owed via the banked hour's flexible working scheme.
- Banked hours will be reviewed with each 4-week roster.
- Redeployment of Intensive Care staff should use best practice principles as per this document, to avoid inherent risks to patient safety with the movement of staff to an unfamiliar area without the appropriate skillset.
- Many Intensive Care staff do not have relevant skills to work on a general ward environment.
- A record of staff redeployment will be kept on the unit and reviewed 6 monthly.
- Staff with a pre-arranged flexible working agreement may be able to opt into the banked hour's scheme depending on the level of flexibility.
- Staff with fixed Rota's will not be eligible to bank hours.
- In times of low occupancy staff will be offered the opportunity to bank hours.
- Redeployment of Intensive Care staff should only be considered to cover short term unplanned need using best practice principles to balance the risk across the organisation.

- A fluid working scheme to enable staff to use hours flexibly responds to the changing occupancy of the Intensive Care Unit. Staff will be able to bank hours at times of low demand and pay hours back at peak times or within 4 weeks whichever is sooner.

3. Self-sufficient staffing

3.1 Banking hours

- The decision to allow staff to bank hours will lie with the Nurse in charge for Intensive Care.
- The decision to permit staff to bank hours will be concluded after an assessment of patient acuity, nursing competency, and potential admissions highlighted by the Medical team within the Intensive Care Unit. Discussion will also take place with the duty intensivist. There will always be a minimum of 2 senior staff (Band 6 / 7) on duty with one of these being a supernumerary Nurse in charge.
- Staff can opt to bank part or the entire shift.
- Any member of staff on sickness monitoring at stage 2 or above will be temporarily excluded from the scheme until the monitoring period is over. The balance of hours will remain static for the monitoring period.
- A maximum of 11.5 hours can be banked.

3.2 Repaying banked hours

- It will be the decision of the Nurse in charge to call staff in to work. Intensive Care staff will not be called in to work in an area other than Intensive Care neither will staff be called into work on Intensive Care to release another member of staff to go to the ward areas.
- When the Rota is created outstanding hours may be used for any staff who owe banked shifts. This will mean some members of the team will work 14 shifts on a 4-week roster period. Adequate rest days post nights will be maintained to ensure staff wellbeing.
- Banked hours should not be offered to an RN if agency staff have been booked to cover a shift. In circumstances where the unit's acuity has changed a decision should be taken to either cancel agency staff, recognizing this may incur a cancellation fee or preferentially reassign the agency staff to another environment in discussion with the late Senior Nurse or the Clinical Site Management team.
- Staff will be expected to review the off-duty numbers to identify gaps when hours can be repaid and commit to paying back the hours within 4-6 weeks. This may mean taking our staffing numbers up on another shift.

- At the time of banking hours, if a future shift has less than 12 RN's assigned or less than HCA's working on it staff will be able to volunteer to repay their hours on that shift.
- Staff may opt to take AL to cover the banked hours.
- Intensive Care staff will pay back hours owed to them within 6 weeks. In exceptional circumstances, any shifts not paid back to the unit will be rostered on the next Rota or entered as annual leave depending on the needs of the unit.
- The unit Administrator or Clinical Nurse Manager will keep account of hours owed via a paper copy (appendix 1) and via the Rota. This will be updated weekly. To ensure accuracy this will be documented on the record sheet with the off duty.

4.0 Redeployment of Intensive Care Staff

4.1 At times there may be a requirement to move nursing staff from one area to another to minimise risk to patient care standards. In principle this should only be required to meet short term (<12 hrs.), unplanned need and should balance the relative risks of ensuring patient safety across the organisation.

4.2 The request to move staff from Intensive Care to another ward area may be made by the Senior Nurse on call or the Clinical Site Management Team.

4.3 In hours (0700 -1700) the approval or rejection of the request will be made by the ICU Clinical Matron. Out of hours this decision will be made by the Senior Nurse on call in consultation with the nurse in charge for ICU. Their decision will be upheld.

4.4 Consideration will be given to the unit's acuity, staff competency, individual staff wellbeing, personal circumstances and training needs. It is imperative for junior nurses, at times, to work with experienced Intensive Care staff, caring for complex critically ill and unstable patients, ensuring their competencies are completed in a timely fashion in accordance with the Guidelines for the Provision of Intensive Care Services. (GPICS 2.1).

At times HCSWs are also required to work with an individual Registered practitioner to support at the bedside with complex patients and when a Registrant has 2 complex L2 patients.

4.5 The Intensive Care unit will never deviate from normal GPICS nurse staff ratios, to enable staff to be moved to the areas agreed within this SOP.

5.0 Considerations & responsibilities

5.1 Emergency admissions and patient deterioration are not predictable and therefore the number of nurses and HCSWs on shift should safely allow for flexibility to respond to changes in patients' clinical conditions the unit's activity and demands.

5.2 There are significant risks associated with moving specialised staff to non-specialised areas:

- Intensive Care nursing staff may have little or no experience working as qualified nurses, or HCSWs on general wards.
- Intensive Care uses a different view of the electronic patient record to that on general wards therefore staff are increasingly unfamiliar with the documentation used on the wards.
- Direct correlation has been established between nurse staffing levels in Intensive Care and the incidence of adverse events.
- Reduction in staffing numbers will impact on the opportunity for junior nurses to work with experienced staff caring for more complex patients, allowing them to complete and sign off mandatory national competencies This is essential to develop a competent workforce and meet the GPICs 2 standards.
- Each occasion that staff members are moved should be clearly risk assessed and monitored, and if recurrent, should be investigated to see if remedial or supportive action is required.

5.3 Allowing Intensive Care to manage their staffing flexibly helps them to provide a better resourced service and benefits nurses allowing them to work in their chosen specialty. Moving staff to a different area reduces the ability to provide a flexible workforce providing safe staffing levels in Intensive Care.

6.0 Best Practice Recommendations

6.1 The following principles should apply when considering the movement of nursing staff from intensive care to a different clinical area:

Organisation (Person responsible for decision to move staff)
• Risk assessment completed and process followed. (Appendix 2) • Completion of Appendix 4 on arrival to the ward • If a member of staff has opted not to bank hours and has decided to work, they can be redeployed to a suitable area • If a member of staff from the unit's core staffing is redeployed i.e. floating nurse, they must be able to return to ICU to take an emergency admission in a timely manner • Registered Intensive Care staff should be moved to the areas where they have shared skills and is within their scope of practice • HCSW's may move to other general ward area's if the unit allows. The decision to move HCSW's will also be at the discretion of the Senior Nurse on call in conjunction with the ICU Nurse in Charge. • Registered Intensive Care staff must not be asked to take charge of another area • Ensure staff member is not asked, or expected, to provide care outside their area of competence; in particular, discharge planning and drug administration rounds may be an area of concern in an unfamiliar care area
Area (Senior Nurse or NIC for Intensive Care)
• Risk assessment of unit acuity and skill mix completed, and process followed. • Redeployment of agency staff first (if able to do so). • Monitor occurrence and staff redeployed as per Appendix 3. • Intensive Care Nurse Trainees should not be moved as they need to complete national competencies whilst still ensuring safe skill mix remains on the unit. • Provide support for staff members who have been moved.
Individual
• Work within area of competence. • Complete orientation checklist on arrival to the new ward. (Appendix 4) • Access training when available i.e. induction opportunities to the above redeployment areas. • Report any concern initially to the Nurse in Charge on the ward of redeployment to seek resolution and escalate to the senior nurse on call or Site Management if no resolution is found.

7.0 REFERENCES

7.1 **Critical Care Networks National Nurse leads (CC3N)** 2018 Best Practice Principles to Apply When Considering Moving Critical Care Nursing Staff to a Different Clinical Care Area. CC3N.

7.2 G Hitchman 2017 Self Sufficient Staffing Model. RUH, Bath

7.3 www.ics.ac.uk. (n.d.). Guidelines for the Provision of Intensive Care Services Version 2.1. [online] Available at: https://www.ics.ac.uk/Society/Guidelines/GPICS/Society/Guidance/GPICS_Version_2.1.aspx?hkey=5dda1ac0-eec7-4b9c-881f-e72f4882d639 [Accessed 12 Aug. 2022].

8.0 Appendix 1 – Self Sufficient Staffing Model Record

Date	Name	Banked Hours	Availability / Date paid back	Risk assessment completed

Appendix 2 – Intensive Care Staffing Status / Risk Assessment

CCNN Critical Care Networks-National Nurse Leads

To be completed by co-ordinator for all movement of staff to any ward

Date & Time of request:		**Ward area that is short staffed:**	
Name of person requesting staff move from CC			
Person completing form (Senior Nurse on duty) **Name & Designation:**			
Number of Staff on duty in Intensive Care			
Registered			
Non-registered			
Level of Care for patients on the unit (number of patients)			
Level	**Number of Patients**	**Calculate**	**Number of nurses required**
Level 3 (1:1)		=	
Level 2 (1:2)		Divide by 2	
Delayed Discharges:			
Level 1			
Level 0			
		Co-ordinator	
		Units with greater than 10 beds + 1 member of staff	
		TOTAL	
Total number of Nurses on duty			
Total number of Nurses required for dependency			
+/- enough resource **(as per National Safe Staffing for Intensive Care)**			
Other factors to be considered: Barrier nursed patients; confused / agitated patients; Patients on renal replacement; complex patient / family.			
Other Action(s) already taken / to be taken:			
Name of Senior Nurse requesting staff informed:			**Time:**

Below required standards Contact Duty matron and directorate manager to seek support from other areas	**Equal** There is **insufficient** capacity to support other areas of the organisation	**Above required standards** Intensive Care staff can offer support to other areas of the organisation nurse **must be able to return** at short notice (<1hour) as required by shift leader

Appendix 3 – Record of staff redeployed from Intensive Care

Date	Time	Name	Moved to:	Number of hours	Risk assessment completed	Staff returned to unit When required Comments:

Appendix 4 – Ward orientation checklist for Intensive Care staff supporting other area

Intensive Care is a different working environment to your clinical area; roles and responsibilities vary greatly. Due to this, Intensive Care nurses are best placed in supportive clinical roles so please discuss the needs of the department with your Intensive Care colleague before allocating roles.

1. Orientation to ward
   - Drug & fluid storage/Medication trolley
   - Store cupboards
   - Key access/policy. Relevant access codes.
   - Staff Room
   - Bathroom
   - Nurses station & telephones

2. Details of expected role (e.g. Observations/medication/personal care)

3. Cardiac arrest system and defib location ☐

4. Unfamiliar equipment check ☐

5. Infection control issues ☐

6. PPE ☐

7. Fire exit, alarms, equipment and evacuation procedure ☐

8. Dr's/outreach contact numbers ☐

Ward……

Signed ..... (Nurse in charge)

Date…….

Signed ..... (Intensive Care Nurse /HCSW)

Copy given to Nurse in charge

With Thanks,
Intensive Care Senior Team